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GP's (Powell, 2016). Previous studies raised doubts about the ability and/or the willingness of clinicians to assume such managerial roles (Dixon and Ham, 2010) and follow-on assessments indicated that the transition was disruptive and expensive. Paton (2014) valued the cost of the reorganisation at over £3 billion, which critics said could have been used to provide care to patients. The Act also brought in Section 75 regulations mandating commissioners tender care to the open market, which Hunter (2008) expected to increase the commercialisation of NHS provision.

The financial environment in which this took place was one of austerity. Funds for the NHS were effectively 'ring fenced' and efficiency savings of £20 billion were required under the Nicholson Challenge by 2015 (NHS England, 2014). It had been predicted by Marmot (2010) that public services cuts would have a disproportionate impact on those in lower socioeconomic groups, as the gap between the most and least deprived communities widened

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Task 1: Significant Historical and Contemporary Landmarks in UK Healthcare Provision

1.1. Labour Government Healthcare Policy Reforms (1997–2010)

The National Health Service was widely seen to be underfunded, fragmented and under pressure from the quasi-market mechanisms imposed by the previous Conservative government when Labour took office in 1997 under Tony Blair (Goodair, 2024). The government's first major policy document, *The New NHS: Modern, Dependable* (Department of Health, 1997), made clear that it was moving away from the market ideology, and that GP fundholding was to be replaced by Primary Care Groups and later Primary Care Trusts. Ham (2009) called it a move to reinvigorate the spirit of collaborative planning in the NHS whilst maintaining some of the characteristics of the purchaser–provider divide that had grown under the internal market.

There has been a lot of growth in public spending on health in the early years of the Labour government. The rise of health expenditure as a share of GDP will need to be substantial over the next 20 years if the expected demands are going to be met, as was clearly shown by Wanless (2002), and this directly affected the Labour party's commitment to raise health expenditure from around 6.8% of GDP in 1997 to over 8% in 2007. The injection of resources enabled a significant reduction in waiting times for outpatient appointments (maximum of 13 weeks by 2004) and for having a referral to a treatment pathway (maximum of 18 weeks by 2008, with the vast majority of people achieving this within half that time) (Greener, 2009).

Labour, however, did not completely reject the market system. The government implemented the reform of the quasi-market from around 2002, introducing what Baggott (2004) termed a "reformed quasi-market" which is now characterised by patient choice, payment by results and an increased number of independent sector treatment centres. Klein (2013) said that this was a significant ideological backdown, since Labour had been against competition and now embraced it as a mechanism of efficiency. The implementation of the foundation trusts under the Health and Social Care (Community Health and Standards) Act 2003 also brought financial autonomy to NHS organisations, a development which some, including Pollock (2004), argued was a form of creeping privatisation of public health services.

1.2. The Conservative/Liberal Democrat Coalition Health Agenda (2010–2015)

By most clinical indicators, the NHS at the time of the Coalition's election in May 2010 was better than at any other time in its history (Klein and Powell, 2025). But the new government acted quickly to get in place what would be one of the most controversial health measures in the history of post-war Britain. The vision of GP-led commissioning, the demise of Primary Care Trusts and Strategic Health Authorities and the establishment of an independent NHS Commissioning Board was outlined in the White Paper *Equity and Excellence: Liberating the NHS* (Department of Health, 2010). Timmins (2012) reported that the next Health and Social Care Act 2012 was a "structural upheaval of extraordinary proportions" with no real electoral support, and that there was "bitter opposition from clinical bodies, trade unions and patient advocacy groups".

The main feature of the Act was the creation of Clinical Commissioning Groups (CCGs) and the transfer of responsibility for commissioning the majority of hospital and community services from the NHS Commissioners to GPs (Powell, 2016). Previous studies raised doubts about the ability and/or the willingness of clinicians to assume such managerial roles (Dixon and Ham, 2010) and follow-on assessments indicated that the transition was disruptive and expensive. Paton (2014) valued the cost of the reorganisation at over £3 billion, which critics said could have been used to provide care to patients. The Act also brought in Section 75 regulations mandating commissioners tender care to the open market, which Hunter (2008) expected to increase the commercialisation of NHS provision.

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1.3. Recent Proposals in Healthcare Policy and Their Potential Impact on Service Users

The structural reorganisation of the NHS has been replaced by a focus on integrated models of care, as a result of NHS England's Five Year Forward View (NHS England, 2014). It set out new care models, including Multispecialty Community Providers and Primary and Acute Care Systems, to break down the barriers between primary, secondary and social care (Dean et al., 2022). The issue of genuine integration requires not only the structural alignment of professional cultures, but a basic realignment that has long been argued for by Greener (2009), and which is still largely unresolved. The NHS Long Term Plan (NHS England, 2019) then pledged to increase the size of primary care networks and speed up investment in cardiovascular, cancer and mental health services. In the eyes

of service users, these proposals had real promise to increase timely access to mental health services from 150,000 - 530,000 by 2023-2024. However, Fotaki (2014) warned that policy aims always exceed service provision, especially where the provision of services is limited by structural issues, like staffing shortages and inadequate funding for social care. The COVID-19 pandemic revealed serious weaknesses in the capacity of the NHS, with currently waiting lists for elective treatment in excess of 7.6 million by 2023, the highest number of patients on waiting lists in NHS history (NHS England, 2023). The impact on service users has been real: missed diagnosis, poor mental health, and health disparities, which impact most on ethnic minority communities, older people and other minorities.



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